

PATIENT MEDICAL PROFILE: SEEMA SHARMA

General Information:

- Full Name: Seema Sharma
- Age: 45 years old
- Gender: Female
- Nationality: Indian
- Primary Language: Hindi, English
- Contact Number: +91 98765 43210
- Emergency Contact: Rajesh Sharma (Spouse) - +91 98765 43211

Clinical Presentation:

- Chief Complaint: Severe retrosternal chest pain radiating to the left arm and jaw. Accompanied by heavy sweating (diaphoresis) and mild shortness of breath (dyspnea).
- Duration: Persistent for the past 2 hours.
- Severity: 8/10 on the pain scale, described as a heavy pressure or squeezing sensation.

Medical History:

- Chronic Conditions: Hypertension (diagnosed 5 years ago), Type 2 Diabetes Mellitus (diagnosed 8 years ago), Hyperlipidemia.
- Family History: Father had a myocardial infarction (heart attack) at age 52.
- Surgical History: Cholecystectomy (gallbladder removal) in 2018.

Current Medications:

- Telmisartan 40 mg orally once daily (for Hypertension)
- Metformin 500 mg orally twice daily with meals (for Diabetes)
- Atorvastatin 20 mg orally once daily at bedtime (for Cholesterol)

Allergies:

- Penicillin (causes mild skin rash/hives)

Triage Vital Signs:

- Blood Pressure: 148/92 mmHg (Elevated)
- Heart Rate: 98 bpm (Regular, borderline tachycardia)
- Respiratory Rate: 20 breaths per minute
- Temperature: 98.4°F (36.9°C) (Afebriles)
- Oxygen Saturation (SpO2): 94% on room air

Triage Assessment & Emergency Department Protocol:

- Suspected Diagnosis: Acute Coronary Syndrome (ACS) / Suspected ST-Elevation Myocardial Infarction (STEMI).
- Recommended ESI Triage Level: ESI Level 2 (High risk, potential active myocardial infarction, requires immediate intervention).
- Required Immediate Interventions (within 10 minutes):
 1. Obtain a 12-lead Electrocardiogram (ECG) and show it to the physician immediately.
 2. Initiate cardiac monitoring.
 3. Establish intravenous (IV) access and draw blood for cardiac enzymes (Troponin I/T), CBC, CMP, and coagulation profile.

4. Administer Aspirin 325 mg orally (chewable).
 5. Administer supplemental oxygen if SpO2 drops below 90%.
- Handoff Instructions: Keep patient in a reclined position. Monitor vitals every 15 minutes. Prepare for immediate transfer to the cardiac catheterization lab if ECG shows ST-elevation. Keep patient NPO (nothing by mouth).